

Policy on Infection Prevention and Control Programme

«Hospital Name»

Document control

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Prepared by	Infection Prevention and Control Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

The IPC programme at «Hospital Name» is comprehensively documented as an IPC Manual, directed at identified high-risk activities and areas, maintained by a designated officer and nurse(s) under a multi-disciplinary committee, assessed with a validated tool at least annually, and extended to the community.

1. Purpose

This policy explains how «Hospital Name» documents its infection prevention and control programme as a written IPC Manual, identifies high-risk activities and areas, reviews and updates the programme at least annually using a validated assessment tool, guides it through a multi-disciplinary IPC committee that meets at least monthly, and runs it day to day through a designated IPC Officer and IPC Nurse(s) supported by link nurses. It also explains how the organisation conducts IEC for the community and participates in managing community outbreaks.

The clinical IPC processes, support-service IPC, HAI-prevention bundles, and surveillance are covered in separate hospital policies.

2. Scope

This policy applies to the IPC Officer, IPC Nurse(s), link nurses, the multi-disciplinary IPC committee, and all staff involved in infection prevention and control activities at «Hospital Name».

3. Policy standards

«Hospital Name» maintains a documented IPC Manual covering the programme's structure, overall aims and objectives, all processes, activities and surveillance procedures, based on scientific knowledge, national/international guidelines, and statutory requirements. High-risk activities and areas are identified and have written guidance. The programme is reviewed and updated at least once a year using a validated assessment tool. A multi-disciplinary IPC committee is constituted with documented composition, a meeting frequency of at least once a month, a defined quorum, and documented minutes; it sets risk-reduction goals and measurable objectives at least annually and reviews them monthly. The IPC team comprises at minimum a designated IPC Officer (a doctor) and IPC Nurse(s) (registered nurses with structured IPC training, at least one per 250 beds), supported by link nurses. An IEC programme reaches the community, and the organisation participates in managing community outbreaks.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not manage infection prevention and control without a documented IPC Manual covering the programme's structure, aims and objectives, all processes, activities and surveillance procedures, based on scientific knowledge, national/international guidelines, and statutory requirements.
- Do not run the IPC programme without identifying high-risk activities and areas and having written guidance to prevent and manage infections for each.
- Do not let the IPC programme go more than a year without review and update; if the annual review finds no opportunities for improvement, document that finding in the IPC committee minutes.
- Do not constitute the IPC committee without documenting its composition, meeting frequency, quorum and minutes; do not let it meet less often than once a month.
- Do not operate without risk-reduction goals and measurable objectives established at least annually and reviewed monthly by the IPC committee.
- Do not designate a non-doctor as the IPC Officer.
- Do not designate as IPC Nurse anyone who is not a registered nurse with additional structured IPC training, and do not staff fewer than one dedicated IPC Nurse per 250 beds.

5. What we do

5.1 Document the IPC programme as a manual

Written infection prevention and control guidance is consolidated into an IPC Manual that covers the structure of the programme, its overall aims and objectives, and all processes, activities and surveillance procedures. The Manual is based on organisational priorities, current scientific knowledge, and national/international guidelines and professional body guidance. Statutory requirements are incorporated where applicable. «Hospital Name» implements

strategies, education, and evidence-based activities based on the Manual to reduce infection risks.

5.2 Identify high-risk activities and areas, and write guidance for them

High-risk activities and high-risk areas across the hospital are identified based on scientific literature and the potential risk of transmission of infections to patients and healthcare providers. Examples of high-risk activities include aerosol-generating procedures, handling blood and body fluids, spills, specimens and sharps, and exposure to contaminated devices, equipment and BMW. Examples of high-risk areas include the OT, ICU, CSSD, cath lab, labour room, laboratory and blood centre. The organisation identifies its own list based on its scope of services. Written guidance to prevent and manage infections is developed for each identified high-risk activity and area.

5.3 Review and update the programme at least annually

The IPC programme is reviewed and updated at least once a year, based on newer scientific literature, infection trends, outbreak prevention developments, and the outcomes of IPC audit processes. If the annual review does not identify any opportunities for improvement, that finding is documented in the IPC committee meeting minutes.

5.4 Assess the programme using a validated tool

«Hospital Name» uses a validated tool to perform a structured assessment of the IPC programme at least annually. Examples of validated tools include the WHO Infection Prevention and Control Assessment Framework at the Facility Level (WHO IPCAF) and the CDC Infection Prevention and Control Assessment Tool for Acute Care Hospitals. The assessment output is documented, gap analysis is completed, and follow-up actions are tracked to closure.

5.5 Constitute a multi-disciplinary IPC committee

A multi-disciplinary IPC committee co-ordinates all IPC activities. Its composition is documented and it includes, where available, representatives from major clinical departments, administration, nursing, the IPC Officer, IPC Nurse(s), CSSD, OT, ICU, and support services. The committee's terms of reference — composition, meeting frequency, and minimum quorum — are defined. The committee meets at least once a month and its minutes are documented. Risk-reduction goals and measurable objectives are established at least annually and reviewed monthly by the committee.

5.6 Maintain an IPC team

An IPC team is responsible for the day-to-day functioning of the IPC programme. The team supports the surveillance process, detects outbreaks, participates in audit activity, and is represented in the IPC committee. The team is staffed according to the organisation's size, risk level, and programme complexity. At minimum, the team comprises the IPC Officer and IPC Nurse(s). IPC Nurses are supported by link nurses — trained clinical staff who actively participate in IPC activities in their work areas.

5.7 Designate an IPC officer

The IPC Officer is a doctor who is knowledgeable in infection prevention and control practices. It is preferable for the IPCO to be a clinical microbiologist or infectious disease specialist; where this is not available, a surgeon or physician may be designated. The IPCO's responsibilities are defined in the IPC Manual. Those requirements related to HR designation are covered in the hospital's other policies.

5.8 Designate IPC nurse(s)

IPC Nurses are designated on the basis of qualification as a registered nurse and completion of additional structured IPC training. Their responsibilities are defined in the IPC Manual and may include: surveillance of HAIs and healthcare-associated organisms; compliance monitoring of hand hygiene, transmission-based precautions, isolation, infection-specific bundles, disinfection and sterilisation; education; and documentation. At minimum, one dedicated IPC Nurse is maintained per 250 beds. Those requirements related to HR designation are covered in the hospital's other policies.

5.9 Run community IEC on infection prevention

«Hospital Name» implements an information, education and communication programme on infection prevention and control for the community. The organisation may work with stakeholders to develop IEC messages covering topics such as hand hygiene, appropriate use of antimicrobials, and use of personal protective equipment.

5.10 Participate in managing community outbreaks

«Hospital Name» coordinates with external agencies, including statutory agencies, to respond effectively to community outbreaks. Internal and external communication channels are defined. Roles and responsibilities for staff during community outbreak response are documented. Staff are trained on their roles.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the IPC programme is resourced and implemented as required by this policy.
Infection Prevention and Control Officer (IPCO)	Owens day-to-day implementation of this policy; leads the IPC team; brings surveillance findings, incidents and audit outcomes to the IPC committee.
Infection Prevention and Control Nurse(s) (IPCN)	Conducts day-to-day IPC activities including surveillance, compliance monitoring and education; supported by link nurses in each clinical area.
IPC Committee	Guides the IPC programme; sets and reviews risk-reduction goals and objectives; meets at least monthly and documents minutes.
Quality Coordinator	Audits this policy; holds training records and

	staff acknowledgements.
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7. Quality monitoring

Monitoring area	What is monitored
IPC Manual	Complete, current, and based on scientific knowledge and national/international guidelines; updated at least annually.
High-risk register	High-risk activities and areas identified; written guidance exists for each.
Annual programme assessment	Validated assessment tool completed at least annually; gap analysis documented; follow-up actions tracked.
IPC committee	Composition, quorum and terms of reference documented; meets at least monthly; minutes documented; goals and objectives set annually and reviewed monthly.
IPC team staffing	IPCO is a doctor; IPC Nurse ratio at least one per 250 beds; link nurses in place in clinical areas; roles defined in manual.
Community IEC	IEC programme implemented; materials or session records available.
Community outbreak participation	External agency contacts defined; staff roles documented; staff trained.

8. Training and staff acknowledgement

The IPC Officer, IPC Nurse(s), link nurses, and IPC committee members shall be familiar with the IPC Manual, the assessment tool process, committee roles, community IEC responsibilities, and the community outbreak response process.

I have read the Policy on Infection Prevention and Control Programme of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to the IPC Officer, IPC Nurse(s), link nurses, the multi-disciplinary IPC committee, department heads, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
AMR	Antimicrobial resistance

BMW	Biomedical waste
CDC	Centers for Disease Control and Prevention
CSSD	Central Sterile Services Department
ICU	Intensive Care Unit
IEC	Information, Education and Communication
IPC	Infection Prevention and Control
IPCC	Infection Prevention and Control Committee
IPCO	Infection Prevention and Control Officer
IPCN	Infection Prevention and Control Nurse
IPCAF	IPC Assessment Framework (WHO tool)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre
WHO	World Health Organization

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.1.a	CORE*	Sections 3 and 5.1 address the documented IPC Manual covering programme structure, aims and objectives, all processes, activities and surveillance procedures, based on scientific knowledge and guidelines.
IPC.1.b	Commitment*	Sections 3 and 5.2 address the identification of high-risk activities and areas and the requirement for written guidance for each.
IPC.1.c	Commitment	Sections 3 and 5.3 address the annual programme review and update, including the documented nil-finding requirement.
IPC.1.d	Achievement	Section 5.4 addresses use of a validated assessment tool with documented gap analysis and follow-up action tracking.
IPC.1.e	Commitment*	Sections 3 and 5.5 address the multi-disciplinary IPC committee: composition, meeting frequency of at least

		monthly, quorum, documented minutes, and monthly review of annually-set objectives.
IPC.1.f	Commitment*	Sections 3 and 5.6 address the IPC team's day-to-day functions, minimum staffing (IPCO and IPCN), and link nurse support structure.
IPC.1.g	Commitment*	Sections 3 and 5.7 address the IPCO designation (must be a doctor) and definition of responsibilities in the IPC Manual.
IPC.1.h	Commitment*	Sections 3 and 5.8 address IPCN designation criteria (registered nurse + structured training), responsibilities defined in the manual, and the minimum staffing ratio of one per 250 beds.
IPC.1.i	Commitment	Section 5.9 addresses the community IEC programme on infection prevention and control.
IPC.1.j	Commitment	Section 5.10 addresses community outbreak participation: external coordination, communication channels, staff roles and staff training.

12. Required Records/Evidence Checklist

IPC Manual — IPC.1.a (CORE*)

- A single documented IPC Manual covering the programme's structure, aims and objectives, all processes, activities and surveillance procedures.
- Evidence the Manual is based on current scientific knowledge and national/international guidelines.
- Version control record showing the Manual has been reviewed at least annually.

High-risk register and written guidance — IPC.1.b (Commitment*)

- Documented list of high-risk activities and high-risk areas identified for «Hospital Name»'s scope of services.
- Written guidance for each identified high-risk activity and area — one document or section per activity/area.

- Evidence the list and guidance were last reviewed in line with the annual update.

Annual review record — IPC.1.c

- Dated review record, or IPC committee minute, confirming the annual update was completed (or, if no gaps were found, documenting that nil-finding explicitly).

Assessment tool output — IPC.1.d (Achievement)

- Completed validated assessment tool (e.g., WHO IPCAF) with scoring and gap analysis, dated.
- Action plan derived from the assessment, with owners and due dates, and records of closure.

IPC committee records — IPC.1.e (Commitment*)

- Committee constitution and terms of reference defining composition, meeting frequency (at least monthly), and minimum quorum.
- Documented minutes of monthly meetings, naming decisions, owners, and due dates.
- Annual risk-reduction goals and measurable objectives document, with evidence of monthly review.

IPC team records — IPC.1.f (Commitment*)

- IPC team composition list showing IPCO and IPCN(s) as the minimum, with a link-nurse register by clinical area.
- Evidence of team's day-to-day functioning: surveillance support, audit participation, and IPC committee representation records.

IPCO designation — IPC.1.g (Commitment*)

- Designation letter for the IPCO naming a doctor with IPC competency.
- IPCO responsibilities defined in the IPC Manual.
- Time-allocation or role-fraction record if the IPCO holds concurrent duties.

IPCN designation and staffing ratio — IPC.1.h (Commitment*)

- Designation letters for all IPC Nurses with qualification (registered nurse) and structured IPC training record.
- IPCN responsibilities defined in the IPC Manual.
- Staffing ratio calculation record showing at least one dedicated IPCN per 250 approved beds.

Community IEC records — IPC.1.i

- Materials, session records, or other evidence of an IEC programme on infection prevention and control delivered to the community.

Community outbreak records — IPC.1.j

- Written external-agency contact list (including statutory agencies).
- Documented internal and external communication path for outbreak response.
- Staff training records on outbreak participation roles.
- Record of at least one drill or real community outbreak response event.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.1.
2. World Health Organization. (2018). <https://www.who.int/publications/i/item/WHO-HIS-SDS-2018.9> Infection Prevention and Control Assessment Framework at the Facility Level (IPCAF).
3. Centers for Disease Control and Prevention. (2016). <https://stacks.cdc.gov/view/cdc/49679> Infection Prevention and Control Assessment Tool for Acute Care Hospitals (Version 1.3.2).

Disclaimer

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